



BEDSIDE
CHILDREN'S HEALTH

**Tell us what's
happening. We'll tell
you what it means.**

THE PROBLEM

It is eleven at night. The child has a fever.

The parent has three options. None of them work.

OPTION 1

Search the internet

Free

Worst-case results, written for nobody in particular. It raises the anxiety without settling anything.

OPTION 2

Wait for a doctor

AED 300, days later

Hours on a helpline, or days for an appointment. The moment the parent needed help has already passed.

OPTION 3

Go to the emergency room

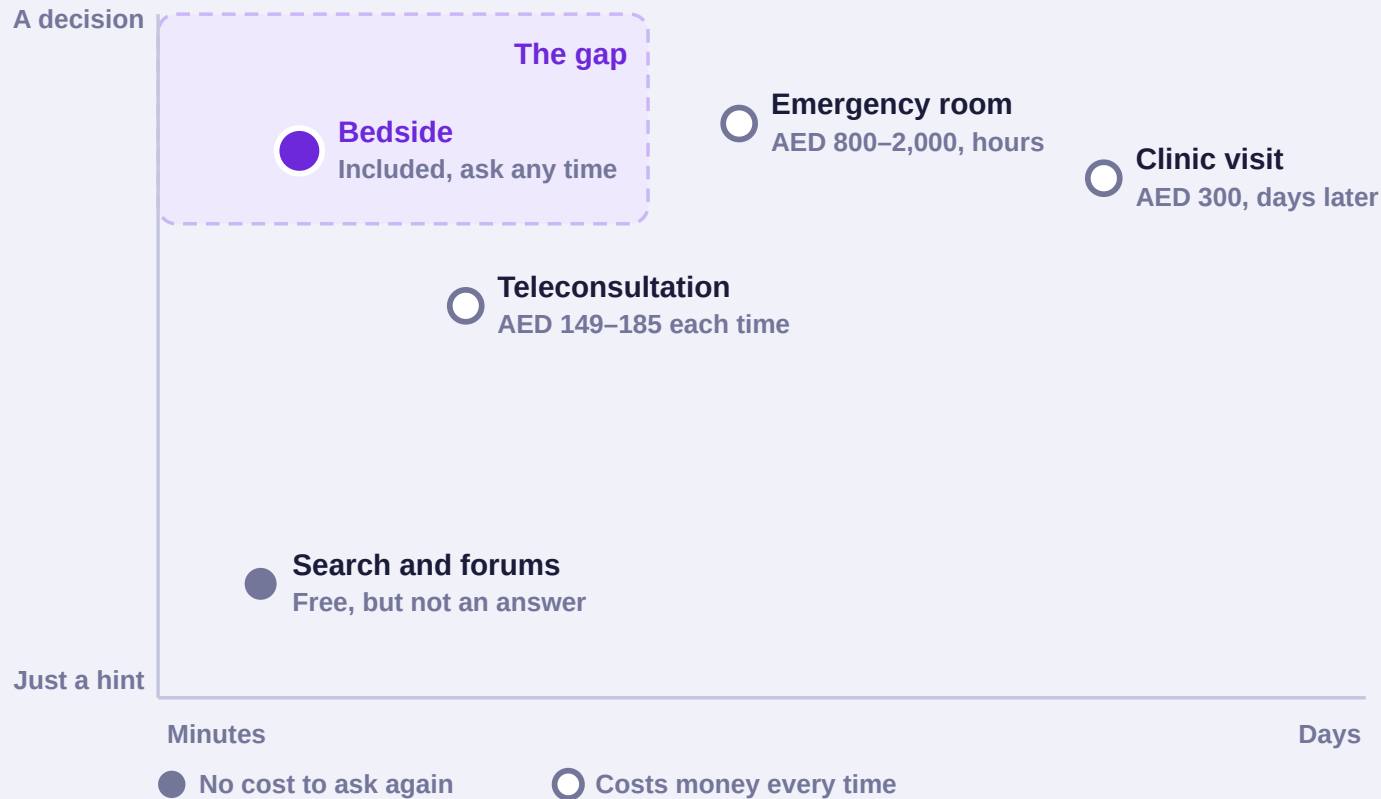
AED 800–2,000

Hours of waiting, often for something that turns out not to be an emergency.

The gap is not a lack of information. **Nothing turns what the parent is looking at into a decision they can act on tonight.**

THE GAP

The fast-and-complete corner is empty.



Everything that answers a parent properly makes them wait, pay, or both. A teleconsultation comes closest, but it is metered, and a phone call leaves nothing written down to check against at three in the morning.

WHAT IT IS

A parent describes what they see. A report comes back.

A short form: which child, what you have noticed, in your own words, how long it has been going on. Minutes later, the same four parts, every time.

PART 1

What you told us

Their own words, read back, so a misunderstanding is visible at once.

PART 2

What it likely means

Plain language, written for that child's age.

PART 3

What to do now

Specific actions for tonight, each with the reason.

PART 4

When to worry

Numbered thresholds, not vague symptoms.

Every report ends with one next step, not an upsell: book fifteen minutes with a paediatrician.

THE REPORT

Four parts, in the same order, every time.

A parent learns where to look. When they are tired at eleven at night, that matters more than anything clever we could do.

- **In your words.** No medical vocabulary required.
- **In plain language.** No jargon unless it is explained in the same sentence.
- **With a clear line.** Thresholds you can check, so you know when to act and when not to.

Your report21:38

A

Amara
2 years 4 months

A fever and a night-time cough, since yesterday

● What you told us

● What it likely means

● What to do now

● When to worry

Book 15 minutes with a paediatrician

WHO IT IS FOR

Parents of children, newborn to fifteen.

What is ordinary at six months is not ordinary at ten years. Every report is written for the age of the child it is about.

 **Under 1**

Babies

Feeding, weight, crying, breathing, temperature. Tighter thresholds, and we say so.

 **1 to 4**

Toddlers

Fevers, coughs, rashes, tummy upsets. The core market: highest frequency, lowest confidence.

 **5 to 11**

School age

Ears, throats, headaches, sleep, days off school, falls.

 **12 to 15**

Teenagers

Growth, mood, sleep, headaches, sports injuries.

WHO BUYS

Usually the most anxious parent in the household. One decision-maker, one payment, the whole family covered.

WHO USES

The other parent, grandparents, and anyone else looking after the children. Access is by household.

EXPANSION

Older children are a retention play, not acquisition. The subscription is already paid.

Everyone else gives information. We give a decision.

THE ALTERNATIVE	WHAT IT LEAVES THE PARENT WITH	WHAT BEDSIDE DOES INSTEAD
Search and forums	Worst-case answers with no way to judge relevance to their own child.	One report, for that child's age and what the parent described.
Symptom checkers	A list of possible conditions, ending in a generic "see a doctor".	A decision and a threshold, not a probability list.
Per-visit telehealth	A fee every time, so parents ration it and wait until they are worried enough.	Flat annual fee, so they ask at the first worry instead of the fifth.
Emergency room	A complete answer, after hours of waiting and AED 800 to 2,000.	Minutes, in writing, and the parent keeps the report.

What is hard to copy: the threshold library, age-banded and written to be checkable, plus a growing record of each child. Anyone can build a symptom form. The defensible asset is the accumulated judgement about where the lines sit.

BUSINESS MODEL

One fee a year. Every child in the home.

AED 399

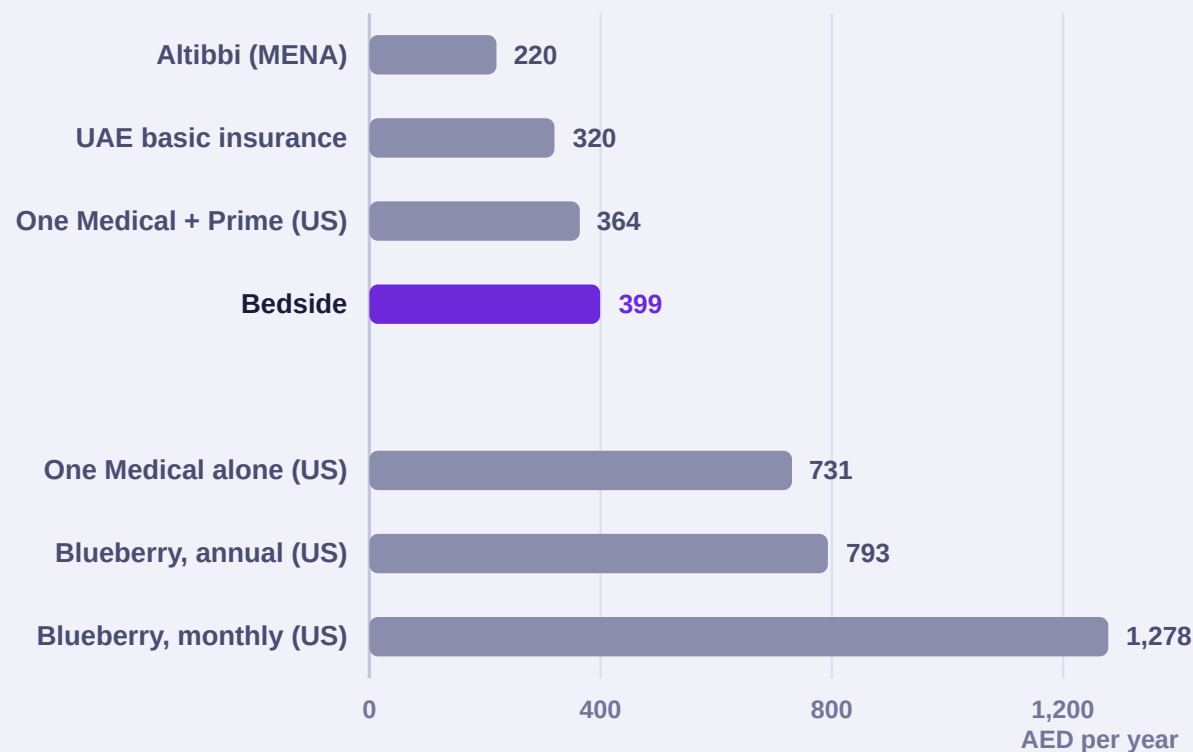
PER HOUSEHOLD, PER YEAR

Children covered	All children under 16 at the address, however many
Assessments	Unlimited
Included	One 15-minute paediatrician call a year, then AED 99 each
Founding cohort	AED 299, locked for life
Billing	Once a year, VAT inclusive

Why flat, not per use. A parent charged per report waits until they are frightened. A parent on a flat fee asks early, which is when guidance is cheapest to give and most useful to receive.

HOW WE SET THAT PRICE

Half of the closest comparable, clear of the commodity apps.



AED 399 is roughly one private paediatric consultation in Dubai, which starts at AED 300. It breaks even against four insurance co-pays, and sits at half of Blueberry Pediatrics, the closest comparable anywhere in the world.

Unlimited doctor calls do not survive UAE clinician rates.

The original model promised unlimited paediatrician calls, included.

Annual fee, per household	AED 399
Cost of one 15-minute paediatrician call, at market rate	AED 150–250
Assumed calls per household, per year	2
Clinician cost alone, before any report is written	AED 300–500

The fix, which keeps the promise intact. Unlimited written assessments, because those are cheap and they are the actual product. One included paediatrician call a year, then AED 99 each. Still well under the AED 149 to 185 a parent pays anywhere else, so the offer stays generous while the expensive resource is metered.

MESSAGING

Everything supports one line.

“Tell us what’s
happening. We’ll tell
you what it means.”

It states the exchange in the parent's own terms and promises exactly one thing.

How we sound. Calm, plain, declarative. Never urgent or alarmed, because an anxious parent does not need us to raise the temperature. Not commercial in tone, because trust is the product.

Calm

Plain

Declarative

Short sentences

No exclamation marks

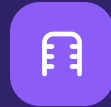
Never alarmed

Never salesy

What we still have to prove.

Everything above is reasoned from market data. None of it is yet evidence from our own customers.

- **Price.** AED 399 is benchmarked, not tested. Run AED 299, 399 and 499 as a live landing-page test before locking it.
- **Cost per report.** Depends on how much clinician review each one needs. It decides whether unlimited is viable at this price.
- **Call attach rate.** One call is included, so it is the largest cost line in the model.
- **Regulatory position.** DHA and DoH licensing for the call, and where guidance ends and a regulated medical device begins.
- **Renewal.** The whole model rests on it, and we have no evidence yet. It is what the founding cohort is for.



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**“Tell us what’s happening.
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Bedside is guidance, not a diagnosis. It does not replace a doctor who
can see your child.